Brahma

Wireframes · 29 Oct 2026

Clinical decision graph for specialists · Decision support, not diagnosis · Synthetic data only

01 - Sign-in

Decision support, not diagnosis. Synthetic patients only. No real patient identifiers.

B

Brahma

Clinical decision graph for specialists. Findings to treatment, every step cited.

Choose a demo account

Each session is tied to the account for the audit trail.

DM **Dr. Maya · Consultant gastroenterologist** — Continue →
Knowledge pack: IBD · v1.3 · first sign-in

DU **Demo urologist · Second specialty** — Continue →
Knowledge pack: Urology stub · v0.1

Or sign in with your hospital account (not in demo build)

Email

name@hospital.org

Password

Sign in

For registered medical practitioners only · Built for the Meta Global AI Developer Hackathon · Theme 2, Indie track

02 - First sign-in - disclaimer

B **Brahma**
Clinical decision graph

Dr. Maya · first sign-in

● STEP 1 Disclaimer | ○ STEP 2 Talk to Brahma | ○ STEP 3 Read the map | ○ STEP 4 Check, edit, sign

Before you start

Brahma walks a curated clinical knowledge graph with you. Every line it shows is tied to a graph node and its source. If the graph has no grounded answer, Brahma says so and does not guess.

Decision support, not diagnosis.
You stay responsible for every clinical decision, prescription and order. Brahma's suggestions are prompts to check, not instructions.

☐ I understand Brahma is decision support and that I remain responsible for clinical decisions.

☐ I'll use synthetic patients only in this demo build. No real patient data.

Your acceptance is recorded once, with the date and knowledge version, in your audit log.

Accept and continue

02b - First sign-in - tour (read the map)

✓ STEP 1 Disclaimer | ✓ STEP 2 Talk to Brahma | ● STEP 3 Read the map | ○ STEP 4 Check, edit, sign

2 · Read the map

The map on the right shows exactly the part of the graph used for this patient. Zoom with − / +, and click any node to see its detail.

- Done or tested
- Test still needed
- ↓ ↑ ! Out of range (uses the patient's sex)
- ✓ Normal
- Path walked
- A branch you're exploring

← Back | Skip the tour | Next

03 - Patients - find or create

B Brahma
Clinical decision graph

Patients | History | Knowledge | Benchmark

IBD pack · Knowledge v1.3 | Dr. Maya (demo) | Sign out

Decision support, not diagnosis. Synthetic patients only. Names are invented and labelled synthetic.

Good morning, Dr. Maya

Open an existing patient to continue their case, or start a new one.

Find a patient

Search by case ID, finding or diagnosis

e.g. P-1042, calprotectin, ulcerative colitis

Recent patients

P-1042	**Lakshmi R. · 34 y · F · UC pancolitis, severe** On Tier 2 (IV steroids) since 29 Oct · day-14 review	◷ Follow-up due today	Visit 1 · 29 Oct →
P-1039	**41 y · diarrhoea 8 wk · calprotectin 420** IBD (working) — awaiting colonoscopy	? Needs data	27 Oct →
P-1031	**52 y · headache 3 wk · visual aura** No grounded answer — outside IBD pack	⊘ Out of knowledge	22 Oct →
P-1024	**63 y · urinary frequency · [PSA value]** Urology stub · [stub pathway outcome]	✓ Plan in place	18 Oct →

New patient

Opens a fresh case with a generated ID. Describe symptoms and results, confirm what was understood, then walk the graph.

+ Create new patient

Next ID: P-1044 · tied to Dr. Maya for the audit trail

04 - Workspace - new patient

B Brahma
Clinical decision graph

Patients | History | Knowledge | Benchmark

IBD pack · Knowledge v1.3 | Dr. Maya (demo) | Sign out

Decision support, not diagnosis. Synthetic patients only. Every line comes from the knowledge graph, never from the model's memory.

? **New case P-1044 · synthetic**
No patient details yet. Describe the patient in the chat; name, age, sex and history fill in here.

NEW CASE P-1044 · SYNTHETIC
Consultation chat

Exit case

B Brahma

Hi Dr. Maya, I'm Brahma. Tell me about the patient: case history, symptoms with how long they've lasted, and any results with values and units. No names or IDs.

Use IBD example

Try a case outside the graph

Describe the patient

e.g. 34-year-old woman, bloody stool on and off for 6 months, Hb 10.2 g/dL, faecal calprotectin 850 µg/g

Send

Decision support, not diagnosis · answers come only from the knowledge graph

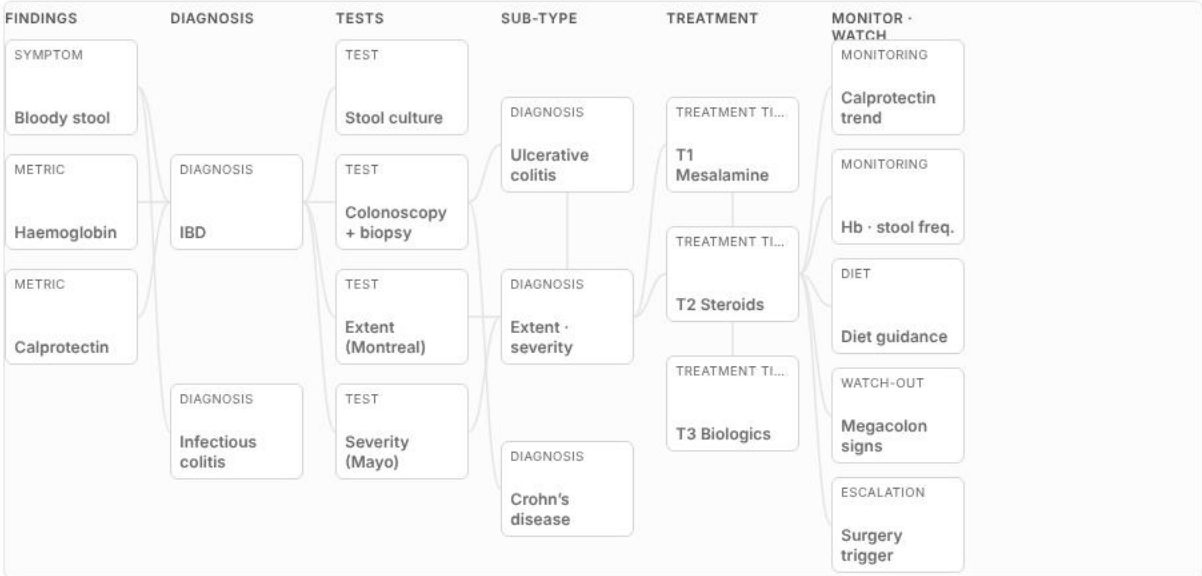
Graph region used · IBD v1.3 · network map

Path walked — Bypassed ---- Not taken --- Exploring

Done / tested | Test needed | ↓↑! Out of range | Normal | Planned

Nothing walked yet. The part of the graph used for this patient will light up here, step by step.

− 100% + Fit 100%



05 - Confirm findings

Decision support, not diagnosis. Synthetic patients only. Every line comes from the knowledge graph, never from the model's memory.

LR **Lakshmi R.** 34 y · Female · 58 kg P-1042 · synthetic
Comorbidities: Hypothyroidism (on treatment) | ▲ Allergy: Penicillin | +3 more

Edit details | Expand ▾

CASE P-1042 · SYNTHETIC
Consultation chat

Exit case

B Brahma

Hi Dr. Maya, I'm Brahma. Tell me about the patient: case history, symptoms with how long they've lasted, and any results with values and units. No names or IDs.

Dr. Maya

34-year-old woman, bloody stool on and off for 6 months, Hb 10.2 g/dL, faecal calprotectin 850 µg/g

B Brahma · form · needs your input

Here's what I picked up. Correct anything before I walk the graph. Durations and units matter.

CONTEXT	Age	34	years
SYMPTOM	Bloody stool	6	months
METRIC	Haemoglobin	10.2	g/dL
METRIC	Faecal calprotectin	850	µg/g

Looks right — walk the graph

+ Add finding

Message the agent — add case history or new results

e.g. "explore infectious colitis", "change dose to 40 mg", "generate prescription", or new history

Send

Decision support, not diagnosis · answers come only from the knowledge graph

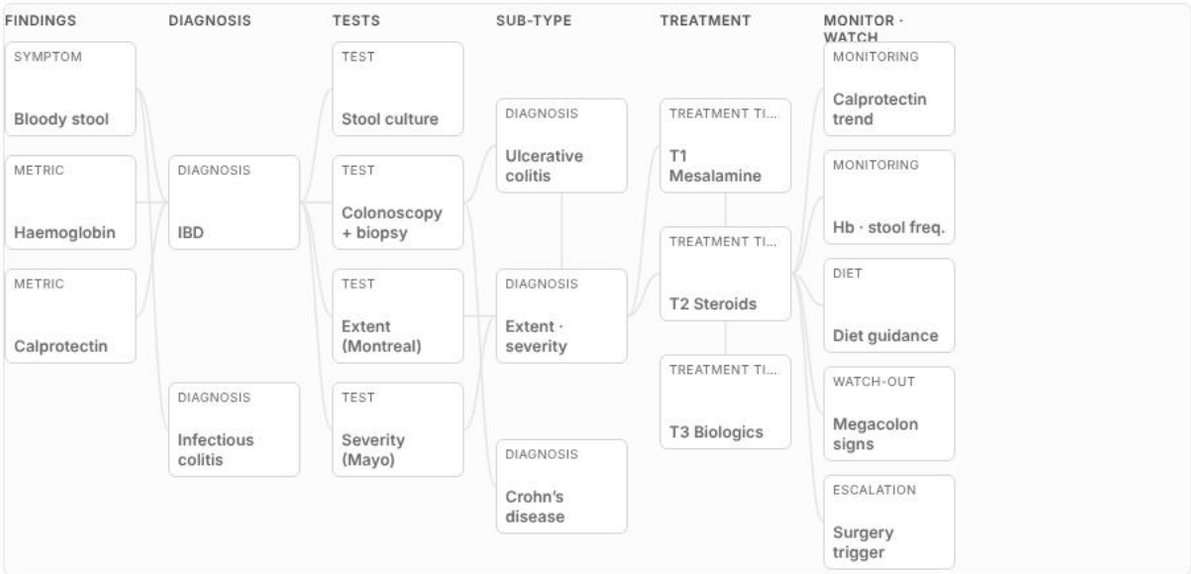
Graph region used · IBD v1.3 · network map

Path walked | Bypassed | Not taken | Exploring

Done / tested | Test needed | ↓ ↑ ! Out of range | Normal | Planned

Nothing walked yet. The part of the graph used for this patient will light up here, step by step.

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06 - Working - path lights up

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CASE P-1042 · SYNTHETIC
Consultation chat
Exit case

Brahma
Hi Dr. Maya, I'm Brahma. Tell me about the patient: case history, symptoms with how long they've lasted, and any results with values and units. No names or IDs.

Dr. Maya
34-year-old woman, bloody stool on and off for 6 months, Hb 10.2 g/dL, faecal calprotectin 850 µg/g

Brahma · form submitted
I read these findings:
- Context · Age 34 years
- Symptom · Bloody stool 6 months
- Metric · Haemoglobin 10.2 g/dL
- Metric · Faecal calprotectin 850 µg/g — Edit

Dr. Maya
✓ Looks right — walk the graph.

Brahma
Walking the graph… Step 6 of 8
Collecting tests that confirm and refine · watch the canvas →

Brahma · pathway
Pathway IBD pack v1.3 · click a line to see its node

1 · FINDINGS
- Bloody stool, on and off for 6 months — N1 — ! Present · 6 mo
- Haemoglobin 10.2 g/dL — below reference range — N2 — ↓ 10.2 g/dL
- Faecal calprotectin 850 µg/g — markedly raised — N3 — ↑ 850 µg/g

2 · WORKING DIAGNOSIS
Why: chronic bloody diarrhoea (over 4 weeks) + raised calprotectin + anaemia match the IBD entry pattern. Infectious colitis stays open until stool tests return.
- Inflammatory bowel disease (IBD) — N4

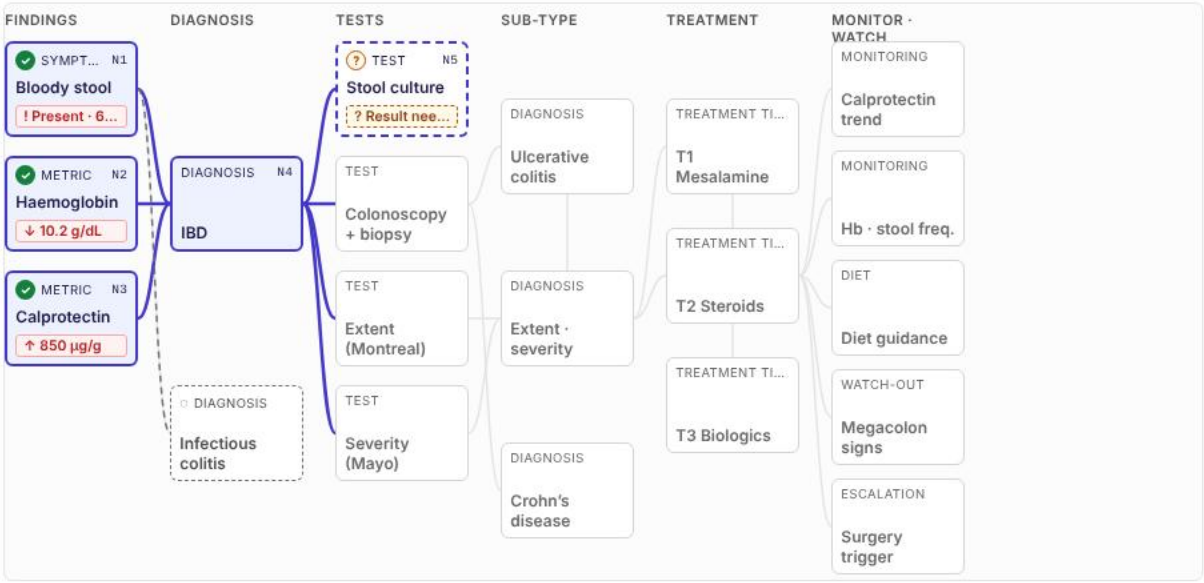
3 · CONFIRM AND REFINE WITH
Each result branches: UC or Crohn's → extent → severity.
- Stool culture and C. difficile toxin — exclude infection first — N5 — ? Result needed

Decision support, not diagnosis · Knowledge v1.3

Message the agent — add case history or new results
e.g. "explore infectious colitis", "change dose to 40 mg", "generate prescription", or new history
Send
Decision support, not diagnosis · answers come only from the knowledge graph

Graph region used · IBD v1.3 · network map
Path walked | Bypassed | Not taken | Exploring
Done / tested | Test needed | Out of range | Normal | Planned

– 100% + Fit 100%



07 - Needs more data

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Comorbidities: Hypothyroidism (on treatment) | ⚠ Allergy: Penicillin | +3 more

Edit details | Expand ▾

CASE P-1042 · SYNTHETIC
Consultation chat

Exit case

Brahma

Hi Dr. Maya, I'm Brahma. Tell me about the patient: case history, symptoms with how long they've lasted, and any results with values and units. No names or IDs.

Dr. Maya

34-year-old woman, bloody stool on and off for 6 months, Hb 10.2 g/dL, faecal calprotectin 850 µg/g

Brahma · form submitted

I read these findings:

- Context · Age 34 years
- Symptom · Bloody stool 6 months
- Metric · Haemoglobin 10.2 g/dL
- Metric · Faecal calprotectin 850 µg/g — Edit

Dr. Maya

✓ Looks right — walk the graph.

Brahma · pathway

Pathway

IBD pack v1.3 · click a line to see its node

1 · FINDINGS

- Bloody stool, on and off for 6 months — N1 — ! Present · 6 mo
- Haemoglobin 10.2 g/dL — below reference range — N2 — ↓ 10.2 g/dL
- Faecal calprotectin 850 µg/g — markedly raised — N3 — ↑ 850 µg/g

2 · WORKING DIAGNOSIS

Why: chronic bloody diarrhoea (over 4 weeks) + raised calprotectin + anaemia match the IBD entry pattern. Infectious colitis stays open until stool tests return.

- Inflammatory bowel disease (IBD) — N4

3 · CONFIRM AND REFINE WITH

Each result branches: UC or Crohn's → extent → severity.

- Stool culture and C. difficile toxin — exclude infection first — N5 — ? Result needed
- Colonoscopy with biopsy — confirms IBD and separates UC from Crohn's — N6 — ? Result needed
- Extent mapping (Montreal E1–E3) — how much colon is involved — N7 — ? Result needed
- Severity scoring (Mayo / Truelove & Witts) — sets the treatment entry tier — N8 — ? Result needed

Decision support, not diagnosis · Knowledge v1.3

Brahma · form · needs your input

To narrow IBD to a sub-type I need four results (marked ? on the map). Fill them in here, or type them in the chat.

Stool culture · C. diff	Colonoscopy + biopsy
Negative	Continuous from re
Extent	**Severity score**
E3 · Pancolitis	Severe

Send results

Not back yet? Exit; the case resumes here.

Message the agent — add case history or new results

e.g. "explore infectious colitis", "change dose to 40 mg", "generate prescription", or new history

Send

Decision support, not diagnosis · answers come only from the knowledge graph

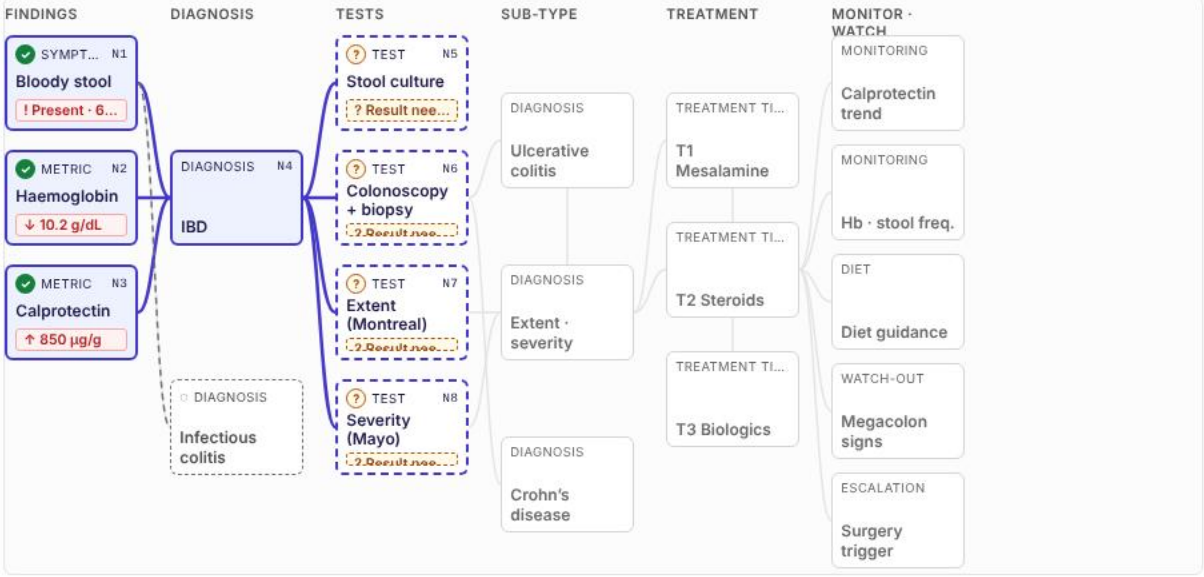
Graph region used · IBD v1.3 · network map

Path walked | Bypassed | Not taken | Exploring

Done / tested | Test needed | ↓ ↑ ! Out of range | Normal | Planned

Paused: the 4 tests marked ? need results. Send them in the chat to continue.

− 100% + Fit 100%



08 - Answered - full pathway

Decision support, not diagnosis. Synthetic patients only. Every line comes from the knowledge graph, never from the model's memory.

LR **Lakshmi R.** 34 y · Female · 58 kg `P-1042 · synthetic`
Comorbidities: Hypothyroidism (on treatment) | ⚠ Allergy: Penicillin | +3 more

Edit details | Expand ▾

CASE P-1042 · SYNTHETIC
Consultation chat

Exit case

Brahma
Hi Dr. Maya, I'm Brahma. Tell me about the patient: case history, symptoms with how long they've lasted, and any results with values and units. No names or IDs.

Dr. Maya
34-year-old woman, bloody stool on and off for 6 months, Hb 10.2 g/dL, faecal calprotectin 850 µg/g

Brahma · form submitted
I read these findings:
- Context · Age 34 years
- Symptom · Bloody stool 6 months
- Metric · Haemoglobin 10.2 g/dL
- Metric · Faecal calprotectin 850 µg/g — Edit

Dr. Maya
✓ Looks right — walk the graph.

Brahma · form submitted
Working diagnosis is IBD N4. To refine it I asked for stool culture, colonoscopy + biopsy, extent and severity.

Dr. Maya
Stool culture: negative · Colonoscopy: continuous from rectum (UC pattern) · Extent: Pancolitis (E3) · Severity: severe

Brahma · pathway

Pathway IBD pack v1.3 · click a line to see its node

SUMMARY

Ulcerative colitis, pancolitis (e3), severe → enters treatment at Tier 2 (corticosteroids)

18 steps | ✓ 18 cited | 0 ungrounded

℞ Generate prescription

◷ Plan follow-up visit

1 · FINDINGS

Bloody stool, on and off for 6 months — N1
! Present · 6 mo

Haemoglobin 10.2 g/dL — below reference range — N2
↓ 10.2 g/dL

Faecal calprotectin 850 µg/g — markedly raised — N3
↑ 850 µg/g

2 · WORKING DIAGNOSIS

Why: chronic bloody diarrhoea (over 4 weeks) + raised calprotectin + anaemia match the IBD entry pattern. Infectious colitis stays open until stool tests return.

Inflammatory bowel disease (IBD) — N4

3 · CONFIRM AND REFINE WITH

Each result branches: UC or Crohn's → extent → severity.

Stool culture and C. difficile toxin — exclude infection first — N5
✓ Negative

Colonoscopy with biopsy — confirms IBD and separates UC from Crohn's — N6
! UC pattern

Extent mapping (Montreal E1–E3) — how much colon is involved — N7
! E3

Severity scoring (Mayo / Truelove & Witts) — sets the treatment entry tier — N8
! Severe

4 · REFINED SUB-TYPE

Ulcerative colitis — N9

Extent: Pancolitis (E3) · severity: severe — N10

5 · TREATMENT PLAN, TIERED

Each tier shows its entry criteria. The patient enters the ladder where their severity fits.

Tier 1 · Mesalamine (5-ASA). Entry: mild to moderate disease — N11
℞ Mesalamine · Oral · [2.4–4.8 g/day] · [Once daily or divided]

Tier 2 · Corticosteroids (IV if severe). Entry: severe disease, or Tier 1 not enough — N12
℞ Methylprednisolone · IV · [60 mg] · [Once daily]
▶ Entry point for this patient

Tier 3 · Biologics — infliximab (Remicade) or ustekinumab (Stelara). Entry: steroid-refractory or steroid-dependent — N13
℞ Infliximab · IV infusion · [5 mg/kg] ≈ 290 mg at 58 kg · [Weeks 0, 2, 6, then every 8 weeks]

6 · MONITOR

Calprotectin — recheck at [interval]; target below [threshold] — N14

Haemoglobin and stool frequency — trend each visit; escalate if [threshold] — N15

7 · DIET

Diet — [guidance from source] — N16

8 · WATCH-OUTS AND SURGERY TRIGGERS

Warning signs — fever, fast heart rate, abdominal distension: urgent review — N17

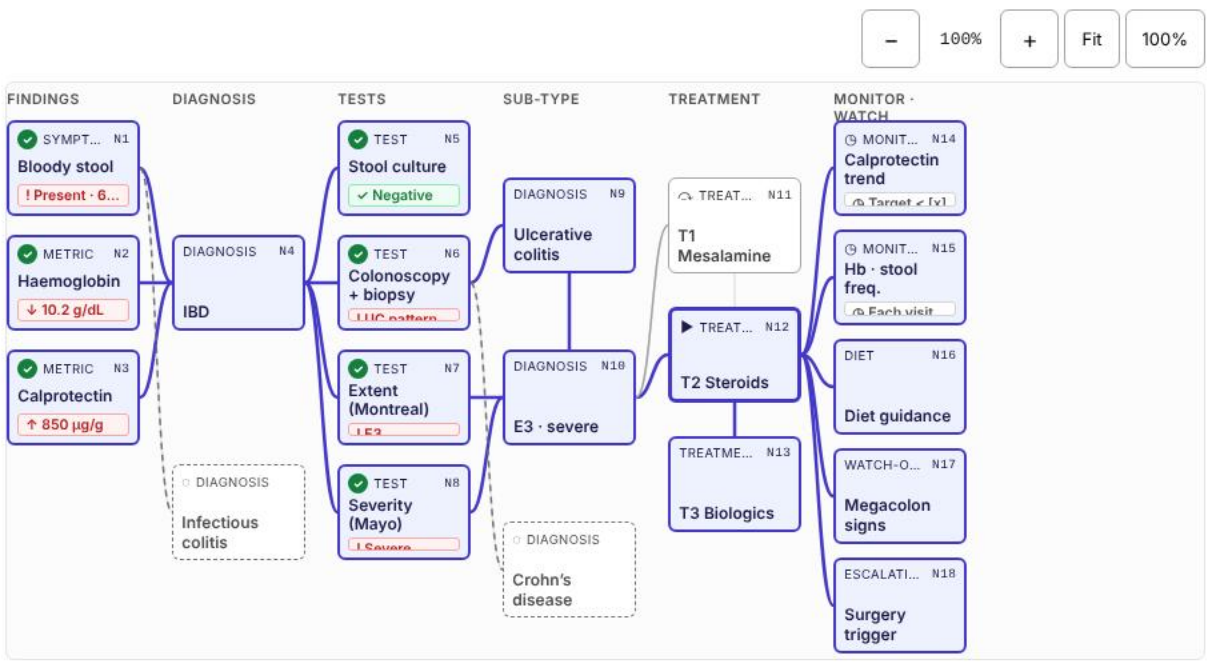
Surgery trigger — no response to IV steroids by day [n] → rescue therapy or colectomy review — N18

Decision support, not diagnosis · Knowledge v1.3

✓ Saved to audit as S-0143 · View in history

Graph region used · IBD v1.3 · network map

Path walked | Bypassed | Not taken | Exploring
Done / tested | Test needed | ↓ ↑ ! Out of range | Normal | Planned



Message the agent — add case history or new results

e.g. "explore infectious colitis", "change dose to 40 mg", "generate prescription", or new history

Send

Decision support, not diagnosis · answers come only from the knowledge graph

09 - Treatment detail panel

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Comorbidities: Hypothyroidism (on treatment) | ⚠ Allergy: Penicillin | +3 more

Edit details | Expand ▾

CASE P-1042 · SYNTHETIC
Consultation chat — Exit case

B Brahma

Hi Dr. Maya, I'm Brahma. Tell me about the patient: case history, symptoms with how long they've lasted, and any results with values and units. No names or IDs.

Dr. Maya

34-year-old woman, bloody stool on and off for 6 months, Hb 10.2 g/dL, faecal calprotectin 850 µg/g

B Brahma · form submitted

I read these findings:

- Context · Age 34 years
- Symptom · Bloody stool 6 months
- Metric · Haemoglobin 10.2 g/dL
- Metric · Faecal calprotectin 850 µg/g — Edit

Dr. Maya

✓ Looks right — walk the graph.

B Brahma · form submitted

Working diagnosis is IBD N4. To refine it I asked for stool culture, colonoscopy + biopsy, extent and severity.

Dr. Maya

Stool culture: negative · Colonoscopy: continuous from rectum (UC pattern) · Extent: Pancolitis (E3) · Severity: severe

B Brahma · pathway

Pathway — IBD pack v1.3 · click a line to see its node

SUMMARY

Ulcerative colitis, pancolitis (e3), severe → enters treatment at Tier 2 (corticosteroids)

18 steps | ✓ 18 cited | 0 ungrounded

℞ Generate prescription

◷ Plan follow-up visit

1 · FINDINGS

Bloody stool, on and off for 6 months — N1
! Present · 6 mo

Haemoglobin 10.2 g/dL — below reference range — N2
↓ 10.2 g/dL

Faecal calprotectin 850 µg/g — markedly raised — N3
↑ 850 µg/g

2 · WORKING DIAGNOSIS

Why: chronic bloody diarrhoea (over 4 weeks) + raised calprotectin + anaemia match the IBD entry pattern. Infectious colitis stays open until stool tests return.

Inflammatory bowel disease (IBD) — N4

3 · CONFIRM AND REFINE WITH

Each result branches: UC or Crohn's → extent → severity.

Stool culture and C. difficile toxin — exclude infection first — N5
✓ Negative

Colonoscopy with biopsy — confirms IBD and separates UC from Crohn's — N6
! UC pattern

Extent mapping (Montreal E1–E3) — how much colon is involved — N7
! E3

Severity scoring (Mayo / Truelove & Witts) — sets the treatment entry tier — N8
! Severe

4 · REFINED SUB-TYPE

Ulcerative colitis — N9

Extent: Pancolitis (E3) · severity: severe — N10

5 · TREATMENT PLAN, TIERED

Each tier shows its entry criteria. The patient enters the ladder where their severity fits.

Tier 1 · Mesalamine (5-ASA). Entry: mild to moderate disease — N11
℞ Mesalamine · Oral · [2.4–4.8 g/day] · [Once daily or divided]

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℞ Methylprednisolone · IV · [60 mg] · [Once daily]
▶ Entry point for this patient

Tier 3 · Biologics — infliximab (Remicade) or ustekinumab (Stelara). Entry: steroid-refractory or steroid-dependent — N13
℞ Infliximab · IV infusion · [5 mg/kg] ≈ 290 mg at 58 kg · [Weeks 0, 2, 6, then every 8 weeks]

6 · MONITOR

Calprotectin — recheck at [interval]; target below [threshold] — N14

Haemoglobin and stool frequency — trend each visit; escalate if [threshold] — N15

7 · DIET

Diet — [guidance from source] — N16

8 · WATCH-OUTS AND SURGERY TRIGGERS

Warning signs — fever, fast heart rate, abdominal distension: urgent review — N17

Surgery trigger — no response to IV steroids by day [n] → rescue therapy or colectomy review — N18

Decision support, not diagnosis · Knowledge v1.3

✓ Saved to audit as S-0143 · View in history

Message the agent — add case history or new results

e.g. "explore infectious colitis", "change dose to 40 mg", "generate prescription", or new history

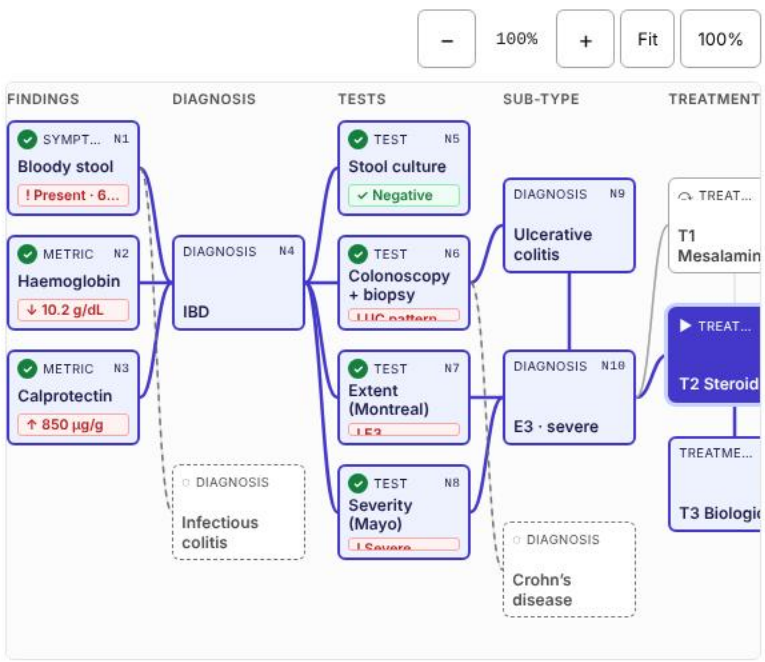
Send

Decision support, not diagnosis · answers come only from the knowledge graph

Graph region used · IBD v1.3 · network map

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Done / tested | Test needed | Out of range | Normal | Planned



TREATMENT TIER · N12

Tier 2 · Corticosteroids

▶ **Entry tier for this patient**

Induction for moderate to severe disease. Severe disease enters the ladder here. Review response at day [n from source].

Treatment plan — Suggested by pack

Type	Corticosteroid
Drug	Methylprednisolone
Route	IV
Dose	[60 mg]
Frequency	[Once daily]
Duration / review	[Review response at day 3]
Pre-start checks	Infection excluded first (stool culture)
Safety	✓ No conflict with recorded allergy (Penicillin) · comorbidities: Hypothyroidism (on treatment) [interaction check per source]

Edit treatment (opens a form in the chat)

Or type it in the chat, e.g. "change dose to 40 mg" or "switch to hydrocortisone". Values in [brackets] are placeholders until the pack's source is loaded.

Source	[UC management guideline, year] · §[section] · Open ↗
Version	`v1.3 · changed 14 Oct 2026`
Node ID	`x2`

Comments (2)

Reviewer A · 14 Oct 2026
Day-[n] review threshold to be checked against the source.

Dr. Maya · 21 Oct 2026
Add a line on the steroid-sparing plan.

Add a comment

Post | Suggest a change

10 - Exploring a side pathway

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Edit details | Expand ▾

CASE P-1042 · SYNTHETIC
Consultation chat | Exit case

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34-year-old woman, bloody stool on and off for 6 months, Hb 10.2 g/dL, faecal calprotectin 850 µg/g

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- Context · Age 34 years
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Dr. Maya
✓ Looks right — walk the graph.

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Working diagnosis is IBD [N4]. To refine it I asked for stool culture, colonoscopy + biopsy, extent and severity.

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Stool culture: negative · Colonoscopy: continuous from rectum (UC pattern) · Extent: Pancolitis (E3) · Severity: severe

Brahma · pathway

Pathway — IBD pack v1.3 · click a line to see its node

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Ulcerative colitis, pancolitis (e3), severe → enters treatment at Tier 2 (corticosteroids)

18 steps | ✓ 18 cited | 0 ungrounded

℞ Generate prescription

◷ Plan follow-up visit

1 · FINDINGS

Bloody stool, on and off for 6 months [N1]
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Haemoglobin 10.2 g/dL — below reference range [N2]
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Faecal calprotectin 850 µg/g — markedly raised [N3]
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Why: chronic bloody diarrhoea (over 4 weeks) + raised calprotectin + anaemia match the IBD entry pattern. Infectious colitis stays open until stool tests return.

Inflammatory bowel disease (IBD) [N4]

3 · CONFIRM AND REFINE WITH

Each result branches: UC or Crohn's → extent → severity.

Stool culture and C. difficile toxin — exclude infection first [N5]
✓ Negative

Colonoscopy with biopsy — confirms IBD and separates UC from Crohn's [N6]
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Extent mapping (Montreal E1–E3) — how much colon is involved [N7]
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Severity scoring (Mayo / Truelove & Witts) — sets the treatment entry tier [N8]
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Ulcerative colitis [N9]

Extent: Pancolitis (E3) · severity: severe [N10]

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Diet — [guidance from source] [N16]

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Decision support, not diagnosis · Knowledge v1.3
✓ Saved to audit as S-0143 · View in history

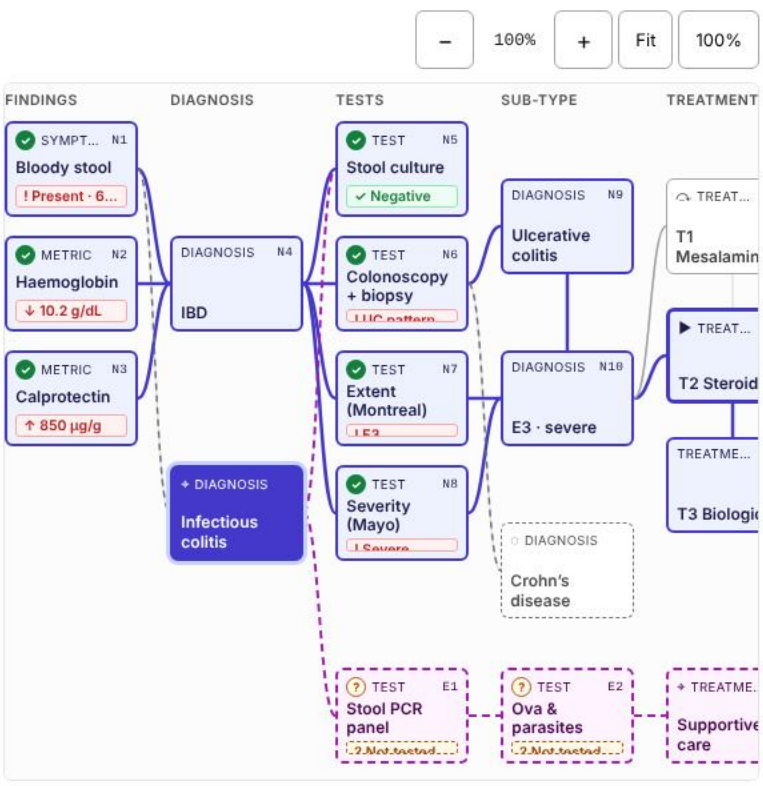
Dr. Maya
Explore: Infectious colitis (from the map)

Brahma
✦ Exploring: Infectious colitis
This is a side branch. It does not change the main plan. To confirm or rule it out, the pack lists: stool PCR panel E1 and ova & parasites E2, both not yet tested. Stool culture is shared with the main path and came back ✓ negative, which makes this branch less likely.
Shown in magenta on the map.
Add these tests to the plan | Stop exploring

Message the agent — add case history or new results
e.g. "explore infectious colitis", "change dose to 40 mg", "generate prescription", or new history
Send
Decision support, not diagnosis · answers come only from the knowledge graph

Graph region used · IBD v1.3 · network map
Path walked — Bypassed ---- Not taken --- Exploring
Done / tested | Test needed | ↓↑ ! Out of range | ✓ Normal | ◷ Planned

Exploring infectious colitis (magenta, dashed). The main plan is unchanged.



DIAGNOSIS · — ×

Infectious colitis

✦ Exploring this branch

Considered and kept open until stool tests return. Less likely given a course over 4 weeks.

Stop exploring

Source	[IBD diagnostic consensus, year] · §[section] · Open ↗
Version	v1.3
Node ID	d2

Comments (0)
No comments yet.
Add a comment
Post | Suggest a change

11 - Edit treatment via chat form

Brahma Clinical decision graph · Patients · History · Knowledge · Benchmark · IBD pack · Knowledge v1.3 · Dr. Maya (demo) · Sign out

Decision support, not diagnosis. Synthetic patients only. Every line comes from the knowledge graph, never from the model's memory.

LR **Lakshmi R.** 34 y · Female · 58 kg P-1042 · synthetic
Comorbidities: Hypothyroidism (on treatment) · ⚠ Allergy: Penicillin

Edit details · Collapse ▲

Sex (sets reference ranges)	Weight (used for mg/kg doses)	Pregnancy status	Allergies
Female	**58 kg**	Not pregnant (test negative)	⚠ Penicillin
Comorbidities		**Current medication**	
Hypothyroidism (on treatment)		Levothyroxine 50 µg once daily	

CASE P-1042 · SYNTHETIC
Consultation chat
Exit case

Brahma
Hi Dr. Maya, I'm Brahma. Tell me about the patient: case history, symptoms with how long they've lasted, and any results with values and units. No names or IDs.

Dr. Maya
34-year-old woman, bloody stool on and off for 6 months, Hb 10.2 g/dL, faecal calprotectin 850 µg/g

Brahma · form submitted
I read these findings:
- Context · Age 34 years
- Symptom · Bloody stool 6 months
- Metric · Haemoglobin 10.2 g/dL
- Metric · Faecal calprotectin 850 µg/g — Edit

Dr. Maya
✓ Looks right — walk the graph.

Brahma · form submitted
Working diagnosis is IBD N4. To refine it I asked for stool culture, colonoscopy + biopsy, extent and severity.

Dr. Maya
Stool culture: negative · Colonoscopy: continuous from rectum (UC pattern) · Extent: Pancolitis (E3) · Severity: severe

Brahma · pathway

Pathway
IBD pack v1.3 · click a line to see its node

SUMMARY
Ulcerative colitis, pancolitis (e3), severe → enters treatment at Tier 2 (corticosteroids)
18 steps · ✓ 18 cited · 0 ungrounded

℞ Generate prescription
◷ Plan follow-up visit

1 · FINDINGS
- Bloody stool, on and off for 6 months — N1 — ! Present · 6 mo
- Haemoglobin 10.2 g/dL — below reference range — N2 — ↓ 10.2 g/dL
- Faecal calprotectin 850 µg/g — markedly raised — N3 — ↑ 850 µg/g

2 · WORKING DIAGNOSIS
Why: chronic bloody diarrhoea (over 4 weeks) + raised calprotectin + anaemia match the IBD entry pattern. Infectious colitis stays open until stool tests return.
- Inflammatory bowel disease (IBD) — N4

3 · CONFIRM AND REFINE WITH
Each result branches: UC or Crohn's → extent → severity.
- Stool culture and C. difficile toxin — exclude infection first — N5 — ✓ Negative
- Colonoscopy with biopsy — confirms IBD and separates UC from Crohn's — N6 — ! UC pattern
- Extent mapping (Montreal E1–E3) — how much colon is involved — N7 — ! E3
- Severity scoring (Mayo / Truelove & Witts) — sets the treatment entry tier — N8 — ! Severe

4 · REFINED SUB-TYPE
- Ulcerative colitis — N9
- Extent: Pancolitis (E3) · severity: severe — N10

5 · TREATMENT PLAN, TIERED
Each tier shows its entry criteria. The patient enters the ladder where their severity fits.
- Tier 1 · Mesalamine (5-ASA). Entry: mild to moderate disease — N11
  ℞ Mesalamine · Oral · [2.4–4.8 g/day] · [Once daily or divided]
- Tier 2 · Corticosteroids (IV if severe). Entry: severe disease, or Tier 1 not enough — N12
  ℞ Methylprednisolone · IV · [60 mg] · [Once daily]
  ▶ Entry point for this patient
- Tier 3 · Biologics — infliximab (Remicade) or ustekinumab (Stelara). Entry: steroid-refractory or steroid-dependent — N13
  ℞ Infliximab · IV infusion · [5 mg/kg] ≈ 290 mg at 58 kg · [Weeks 0, 2, 6, then every 8 weeks]

6 · MONITOR
- Calprotectin — recheck at [interval]; target below [threshold] — N14
- Haemoglobin and stool frequency — trend each visit; escalate if [threshold] — N15

7 · DIET
- Diet — [guidance from source] — N16

8 · WATCH-OUTS AND SURGERY TRIGGERS
- Warning signs — fever, fast heart rate, abdominal distension: urgent review — N17
- Surgery trigger — no response to IV steroids by day [n] → rescue therapy or colectomy review — N18

Decision support, not diagnosis · Knowledge v1.3
✓ Saved to audit as S-0143 · View in history

Dr. Maya
Edit treatment: Tier 2 (from the map)

Brahma · form · needs your input
Edit treatment: Tier 2 · Corticosteroids · prefilled with the pack's suggestion. Change what you need, then apply.

Drug	Route
Methylprednisolon	IV
Dose	**Frequency**
[60 mg]	[Once daily]

Duration / review: [Review response at day 3]

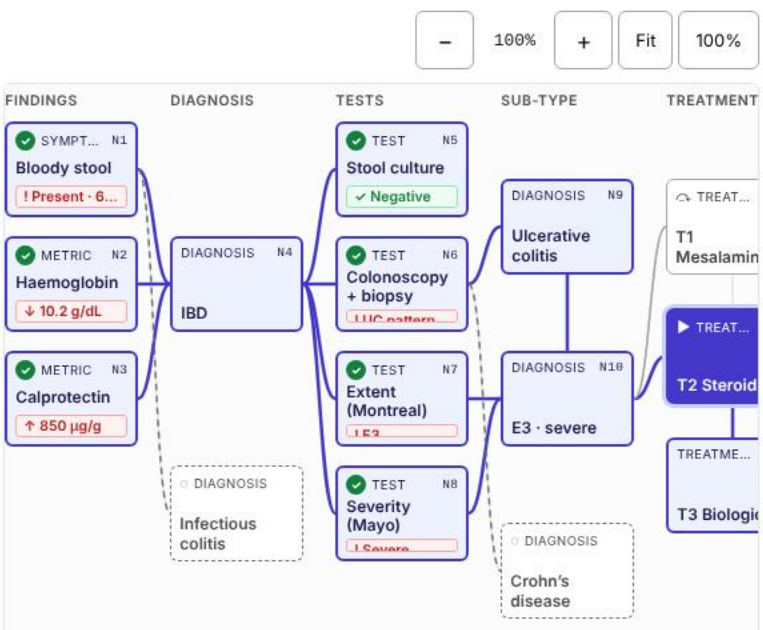
Checks run on apply: allergies (Penicillin), weight 58 kg, comorbidities. Your edit is logged against the suggestion in the audit.

Apply changes · Cancel

Message the agent — add case history or new results
e.g. "explore infectious colitis", "change dose to 40 mg", "generate prescription", or new history
Send

Decision support, not diagnosis · answers come only from the knowledge graph

Graph region used · IBD v1.3 · network map
Path walked · Bypassed · Not taken · Exploring
Done / tested · Test needed · Out of range · Normal · Planned



TREATMENT TIER · N12

Tier 2 · Corticosteroids
▶ Entry tier for this patient

Induction for moderate to severe disease. Severe disease enters the ladder here. Review response at day [n from source].

Treatment plan · Suggested by pack

Type	Corticosteroid
Drug	Methylprednisolone
Route	IV
Dose	[60 mg]
Frequency	[Once daily]
Duration / review	[Review response at day 3]
Pre-start checks	Infection excluded first (stool culture)
Safety	✓ No conflict with recorded allergy (Penicillin) · comorbidities: Hypothyroidism (on treatment) [interaction check per source]

Edit treatment (opens a form in the chat)
Or type it in the chat, e.g. "change dose to 40 mg" or "switch to hydrocortisone". Values in [brackets] are placeholders until the pack's source is loaded.

Source	[UC management guideline, year] · §[section] · Open ↗
Version	v1.3 · changed 14 Oct 2026
Node ID	x2

Comments (2)
Reviewer A · 14 Oct 2026
Day-[n] review threshold to be checked against the source.

Dr. Maya · 21 Oct 2026
Add a line on the steroid-sparing plan.

Add a comment
Post · Suggest a change

12 - Out of knowledge

Brahma
Clinical decision graph

Patients | History | Knowledge | Benchmark

IBD pack · Knowledge v1.3 | Dr. Maya (demo) | Sign out

Decision support, not diagnosis. Synthetic patients only. Every line comes from the knowledge graph, never from the model's memory.

AK **Arun K.** 52 y · Male · 74 kg P-1031 · synthetic

Comorbidities: Type 2 diabetes +1 | ⚠ Allergy: None recorded | +3 more

Edit details | Expand ▾

CASE P-1031 · SYNTHETIC

Consultation chat

Exit case

B Brahma

Hi Dr. Maya, I'm Brahma. Tell me about the patient: case history, symptoms with how long they've lasted, and any results with values and units. No names or IDs.

Dr. Maya

52-year-old, headache for 3 weeks with visual aura

B Brahma · form submitted

I read these findings:

Context · Age 52 years

Symptom · Headache 3 weeks

Symptom · Visual aura Edit

Dr. Maya

✓ Looks right — walk the graph.

B Brahma

No grounded answer

Nothing in IBD pack v1.3 matches these findings (headache, visual aura). The agent does not answer from its own memory, so there is no recommendation.

Nothing has been guessed. Logged to the audit as "out of knowledge".

Edit findings | Suggest knowledge to add

Message the agent — add case history or new results

e.g. "explore infectious colitis", "change dose to 40 mg", "generate prescription", or new history

Send

Decision support, not diagnosis · answers come only from the knowledge graph

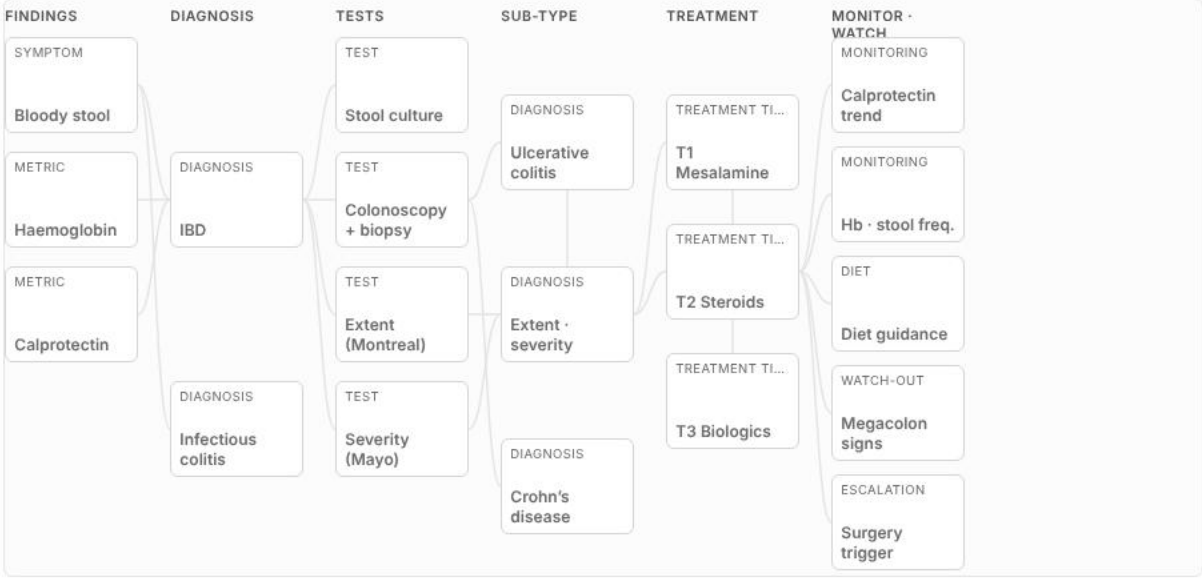
Graph region used · IBD v1.3 · network map

Path walked — Bypassed ---- Not taken --- Exploring

Done / tested | Test needed | ↓↑! Out of range | Normal | Planned

No entry node in IBD pack v1.3 matches these findings, so nothing is lit.

– 100% + Fit 100%



13 - Error

Decision support, not diagnosis. Synthetic patients only. Every line comes from the knowledge graph, never from the model's memory.

LR **Lakshmi R.** 34 y · Female · 58 kg `P-1042 · synthetic`
Comorbidities: Hypothyroidism (on treatment) | ⚠ Allergy: Penicillin | +3 more

Edit details | Expand ▾

CASE P-1042 · SYNTHETIC
Consultation chat

Exit case

B Brahma

Hi Dr. Maya, I'm Brahma. Tell me about the patient: case history, symptoms with how long they've lasted, and any results with values and units. No names or IDs.

Dr. Maya

34-year-old woman, bloody stool on and off for 6 months, Hb 10.2 g/dL, faecal calprotectin 850 µg/g

B Brahma · form submitted

I read these findings:

- Context · Age 34 years
- Symptom · Bloody stool 6 months
- Metric · Haemoglobin 10.2 g/dL
- Metric · Faecal calprotectin 850 µg/g — Edit

Dr. Maya

✓ Looks right — walk the graph.

B System

⚠ The graph service didn't respond

Nothing was answered or guessed. Your messages are kept.

`Ref: [error code] · [timestamp]`

Retry

Message the agent — add case history or new results

e.g. "explore infectious colitis", "change dose to 40 mg", "generate prescription", or new history

Send

Decision support, not diagnosis · answers come only from the knowledge graph

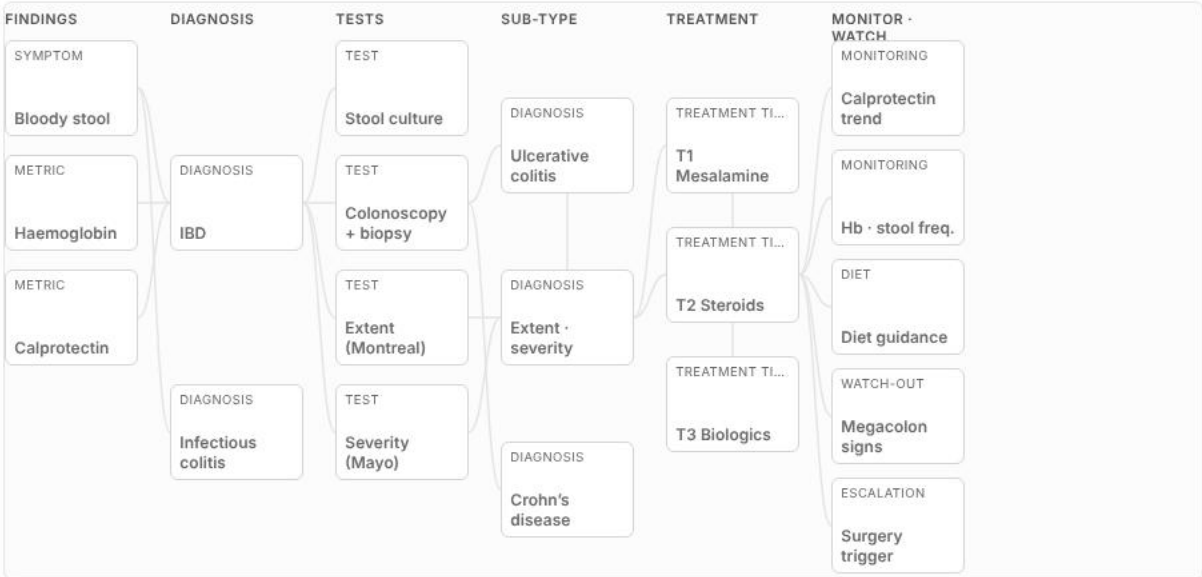
Graph region used · IBD v1.3 · network map

Path walked | Bypassed | Not taken | Exploring

Done / tested | Test needed | Out of range | Normal | Planned

Walk not started: the service is unavailable.

− 100% + Fit 100%



14 - Generate prescription

Brahma
Clinical decision graph

Patients | History | Knowledge | Benchmark

Dr. Maya (demo) Sign out

Decision support, not diagnosis. Brahma drafts from the plan. You review, edit and sign. Demo prescriptions are for synthetic patients only and are watermarked.

LR **Lakshmi R.** 34 y · Female · 58 kg `P-1042` · `synthetic` ⚠ Allergy: Penicillin

← Back to case

℞ Draft prescription

Drafted from the plan · Tier 2

Each line comes from a graph node or the patient card. Untick to leave a line out; edit any field. Generic names are printed in capitals.

☑ **METHYLPREDNISOLONE** N12 Tier 2 · from the plan

Form · route	Dose	Frequency	Duration
Injection · IV	[60 mg]	[Once daily]	[Review on c

Instructions for the patient: Given in hospital.

☑ **LEVOTHYROXINE** Card

Current medication · patient card

Form · route	Dose	Frequency	Duration
Tablet · oral	50 µg	Once daily, t	Continue

Instructions for the patient: Existing medicine. No change.

+ Add medicine

Or ask in the case chat, e.g. "add pantoprazole 40 mg once daily". Brahma checks it against the pack and the card before adding.

Dr. Maya [Surname]
MD, DM (Gastroenterology) · Consultant gastroenterologist
Reg. No. [State Medical Council number]

[Hospital / clinic name]
[Address, city]
[Phone]

Patient: Lakshmi R. (synthetic) **Date:** 29 Oct 2026
Age / sex / weight: 34 y · F · 58 kg **Case ID:** P-1042
Allergy: Penicillin
Diagnosis: Ulcerative colitis, pancolitis (E3), severe · ICD-10 [K51.0, confirm]

℞

1. **METHYLPREDNISOLONE** · Injection · IV · [60 mg]
   [Once daily] · [Review on day 3]
   Given in hospital.
2. **LEVOTHYROXINE** · Tablet · oral · 50 µg
   Once daily, before breakfast · Continue
   Existing medicine. No change.

Investigations

Faecal calprotectin and haemoglobin: recheck at [interval]. Stool culture done 29 Oct: negative.

Advice

Diet: [guidance from source]. Come to emergency at once with fever, a fast heartbeat, a swollen or very painful abdomen, or heavy bleeding.

Follow-up

Response review on 1 Nov 2026 (day 3). Clinic visit on 12 Nov 2026 (day 14).

Generated with Brahma decision support · IBD pack v1.3 · audit S-0143

Not signed · draft

DEMO · SYNTHETIC PATIENT

Safety checks

- ✓ **Allergy** · No conflict with the recorded allergy (penicillin).
- ✓ **Pregnancy** · Not pregnant (test negative), from the patient card.
- ? **Interactions** · Corticosteroid with levothyroxine: [interaction check per source]. Needs your review.
- ✓ **Duplicates** · No two medicines from the same class.
- ? **Doses from source** · 3 value(s) are still [placeholders]. Replace them with the cited source values.

? 3 dose field(s) still hold [placeholders] (dashed amber). In production these come from the cited source.

☐ I have reviewed every medicine, dose and instruction, and I take responsibility for this prescription.

Sign and generate PDF | Save draft

14b - Prescription - signed

Decision support, not diagnosis. Brahma drafts from the plan. You review, edit and sign. Demo prescriptions are for synthetic patients only and are watermarked.

LR **Lakshmi R.** 34 y · Female · 58 kg P-1042 · synthetic ⚠ Allergy: Penicillin

← Back to case

℞ Draft prescription

Drafted from the plan · Tier 2

METHYLPREDNISOLONE N12 Tier 2 · from the plan

Form · route	Dose	Frequency	Duration
Injection · IV	[60 mg]	[Once daily]	[Review on c

Instructions for the patient: Given in hospital.

LEVOTHYROXINE Card

Current medication · patient card

Form · route	Dose	Frequency	Duration
Tablet · oral	50 µg	Once daily, t	Continue

Instructions for the patient: Existing medicine. No change.

+ Add medicine

Or ask in the case chat, e.g. "add pantoprazole 40 mg once daily". Brahma checks it against the pack and the card before adding.

Safety checks

- **Allergy** · No conflict with the recorded allergy (penicillin).
- **Pregnancy** · Not pregnant (test negative), from the patient card.
- **Interactions** · Corticosteroid with levothyroxine: [interaction check per source]. Needs your review.
- **Duplicates** · No two medicines from the same class.
- **Doses from source** · 3 value(s) are still [placeholders]. Replace them with the cited source values.

Dr. Maya [Surname]

[Hospital / clinic name]

Patient: Lakshmi R. (synthetic) **Date:** 29 Oct 2026
Age / sex / weight: 34 y · F · 58 kg **Case ID:** P-1042
Allergy: Penicillin
Diagnosis: Ulcerative colitis, pancolitis (E3), severe · ICD-10 [K51.0, confirm]

℞

1. **METHYLPREDNISOLONE** · Injection · IV · [60 mg]
   [Once daily] · [Review on day 3]
   Given in hospital.
2. **LEVOTHYROXINE** · Tablet · oral · 50 µg
   Once daily, before breakfast · Continue
   Existing medicine. No change.

Investigations

Faecal calprotectin and haemoglobin: recheck at [interval]. Stool culture done 29 Oct: negative.

Advice

Diet: [guidance from source]. Come to emergency at once with fever, a fast heartbeat, a swollen or very painful abdomen, or heavy bleeding.

Follow-up

Response review on 1 Nov 2026 (day 3). Clinic visit on 12 Nov 2026 (day 14).

DEMO · SYNTHETIC PATIENT

Generated with Brahma decision support · IBD pack v1.3 · audit S-0143

✓ e-signed · Dr. Maya
29 Oct 2026 · 10:58 · ID RX-0143-01

✓ Signed. PDF ready and saved to the case audit.

Download PDF | Print | Share with patient (later) | Void and re-issue

15 - Follow-up visit - enter results

Brahma
Clinical decision graph

Patients | History | Knowledge | Benchmark

IBD pack · Knowledge v1.3 | Dr. Maya (demo) | Sign out

Decision support, not diagnosis. Synthetic patients only. Trends compare against ranges picked from the patient card.

LR **Lakshmi R.** 34 y · Female · 58 kg P-1042 · synthetic

UC · pancolitis (E3) · severe | On Tier 2 · methylprednisolone IV since 29 Oct | ⚠ Allergy: Penicillin

Visit 3 · day 14 · 12 Nov 2026 | Exit case

Follow-up chat

B Brahma

Welcome back. This is Lakshmi's day-14 review on Tier 2 (IV methylprednisolone). The pack judges response on calprotectin, Hb, stool frequency and bleeding N14–15. Enter today's results and I'll compare them with the last two visits.

B Brahma · form · needs your input

Haemoglobin (g/dL): 11.4

Faecal calprotectin (µg/g): 310

Stool frequency (per day): 4

Rectal bleeding: Streaks, less thar

Send results

Message Brahma

e.g. "she reports less urgency", "compare with visit 1", "generate prescription"

Send

Treatment timeline

✓ 29 Oct · Visit 1
Diagnosis · Tier 2 started

✓ 1 Nov · Day 3
IV steroid response check

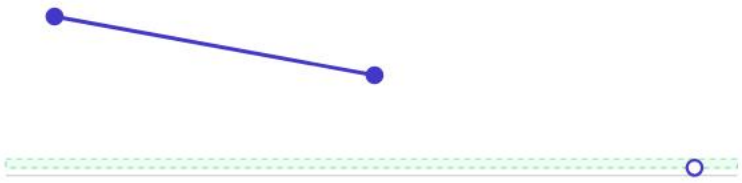
✓ 5 Nov · Visit 2
Partial response

● 12 Nov · Visit 3
Day-14 review · today

Faecal calprotectin

? Today not entered

Target < 50 µg/g [per source] · lower is better



Haemoglobin

? Today not entered

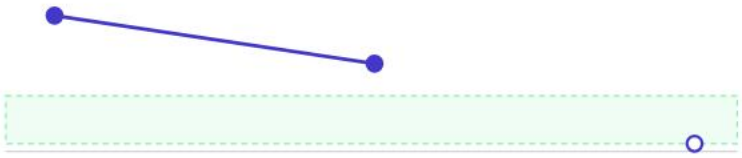
Range 12.0–15.5 g/dL for female, picked from the patient card · higher is better



Stool frequency

? Today not entered

Target ≤ 3 per day [per source] · lower is better



15b - Follow-up visit - assessment and trends

Decision support, not diagnosis. Synthetic patients only. Trends compare against ranges picked from the patient card.

LR **Lakshmi R.** 34 y · Female · 58 kg `P-1042 · synthetic`
UC · pancolitis (E3) · severe | On Tier 2 · methylprednisolone IV since 29 Oct | ⚠ Allergy: Penicillin

Visit 3 · day 14 · 12 Nov 2026 | Exit case

Follow-up chat

Brahma

Welcome back. This is Lakshmi's day-14 review on Tier 2 (IV methylprednisolone). The pack judges response on calprotectin, Hb, stool frequency and bleeding N14–15. Enter today's results and I'll compare them with the last two visits.

Brahma · form submitted

Haemoglobin (g/dL)	Faecal calprotectin (µg/g)
11.4	310
Stool frequency (per day)	**Rectal bleeding**
4	Streaks, less thar

Change results

Brahma · assessment

RESPONSE TO TIER 2 AT DAY 14

‹ Partial response

- Calprotectin 310 µg/g: 64% lower than at diagnosis (850), target < 50.
- Haemoglobin 11.4 g/dL: rising but still below the female range (12.0–15.5).
- Stool frequency 4/day (from 8); bleeding: streaks, less than half the time.

NEXT STEP PER THE PACK N12 · N13

Continue Tier 2 and recheck at day [n]. If there is no further improvement by then, Tier 3 (biologics) entry criteria are met.

℞ Renew current prescription

Open the case map

Decision support, not diagnosis · saved to the audit as visit 3 · S-0151

Message Brahma

e.g. "she reports less urgency", "compare with visit 1", "generate prescription"

Send

Treatment timeline

✓ 29 Oct · Visit 1	✓ 1 Nov · Day 3	✓ 5 Nov · Visit 2	● 12 Nov · Visit 3
Diagnosis · Tier 2 started	IV steroid response check	Partial response	Day-14 review · today

Faecal calprotectin

↓ Improving · still above target

Target < 50 µg/g [per source] · lower is better



Haemoglobin

↑ Improving · still below range

Range 12.0–15.5 g/dL for female, picked from the patient card · higher is better



Stool frequency

↓ Improving · still above target

Target ≤ 3 per day [per source] · lower is better



16 - Session history - audit

Decision support, not diagnosis. Synthetic patients only. Audit records are read-only.

Session history

Search sessions

Finding, diagnosis or session ID

All | ✓ Answered | ? Needs data | ⊘ Out of knowledge

S-0143 · 29 Oct 2026 · 10:42
34 y · bloody stool 6 mo · Hb 10.2 · calprotectin 850
UC · pancolitis (E3) · severe → Tier 2
✓ Answered · IBD v1.3

S-0139 · 27 Oct 2026 · 16:05
41 y · diarrhoea 8 wk · calprotectin 420
IBD (working) — awaiting colonoscopy
? Needs more data · IBD v1.3

S-0131 · 22 Oct 2026 · 11:18
52 y · headache 3 wk · visual aura
No grounded answer — outside IBD pack
⊘ Out of knowledge · IBD v1.3

S-0124 · 18 Oct 2026 · 09:30
63 y · urinary frequency · [PSA value]
Urology stub · [stub pathway outcome]
✓ Answered · Urology stub v0.1

S-0143 · 29 Oct 2026 · 10:42 · Dr. Maya · IBD v1.3

UC · pancolitis (E3) · severe → Tier 2

✓ Answered · 18 of 18 lines cited

Replay on canvas | Export audit record

1 · INPUTS

"34-year-old, bloody stool on and off for 6 months, Hb 10.2 g/dL, faecal calprotectin 850 µg/g"

Age 34 years | Bloody stool 6 months | Haemoglobin 10.2 g/dL | Faecal calprotectin 850 µg/g | Colonoscopy: UC pattern | Extent E3 | Severity: severe

2 · PLAIN-LANGUAGE WHY

Six months of bloody stool with a high calprotectin and low haemoglobin matched the IBD entry pattern. Colonoscopy showed continuous inflammation from the rectum, which is the ulcerative colitis branch. The whole colon was involved and severity scored severe, so the plan starts at Tier 2 (steroids) rather than Tier 1, with biologics as the next step if steroids fail.

3 · PATH WALKED

N1–3 Findings → N4 IBD → N5–8 Tests → N9 Ulcerative colitis → N10 E3 · severe → N12 Tier 2 · steroids → N13 Tier 3 · biologics → N14–18 Monitor · diet · watch-outs

4 · ANSWER AS GIVEN

- Working diagnosis: IBD
- Refined: ulcerative colitis, pancolitis (E3), severe
- Enter treatment at Tier 2 (corticosteroids); escalate to biologics if steroid-refractory
- Monitor calprotectin, Hb, stool frequency against [thresholds]
- Surgery review if no response to IV steroids by day [n]

5 · SOURCES USED

Source	Nodes cited	Version at time
[IBD diagnostic consensus, year]	N3–N10	v1.3
[UC management guideline, year]	N8, N11–N18	v1.3
[Lab reference ranges]	N2	v1.2

Decision support, not diagnosis · Record is immutable · Signed to Dr. Maya

17 - Knowledge update + versions

Decision support, not diagnosis. Changes never go live directly: every proposal becomes a new version after clinical review.

Suggest a change

Link a source, say what should change and why. It becomes a proposed version with your comment attached.

Node to change

N13 · Tier 3 · Biologics (Treatment tier)

Source

Link (URL) | Upload document

Source URL

https://... guideline or paper

Section or page

e.g. §4.2, p. 18

What should change

e.g. Add [new biologic] as a Tier 3 option with its fit criteria.

Comment (why)

e.g. Updated guideline adds this option for steroid-refractory UC.

Submit as v1.4 proposal Back to node

Proposed version

v1.3 → v1.4 (draft)

STEP 1 ● Proposed | STEP 2 ○ Clinical review | STEP 3 ○ Published

BEFORE · V1.3

Tier 3 · Biologics — infliximab (Remicade) or ustekinumab (Stelara). Entry: steroid-refractory or steroid-dependent.

AFTER · V1.4 DRAFT

Tier 3 · Biologics — infliximab (Remicade) or ustekinumab (Stelara). Entry: steroid-refractory or steroid-dependent. **+ [your described change appears here]**

New source	[linked URL]
Affects	1 node · [n] past sessions would read differently (they keep v1.3)
Proposed by	Dr. Maya · draft, not submitted

Version history · IBD pack

In use now: v1.3

Version	Status	Date	By	Change	Comment
v1.3	● Live	14 Oct 2026	Reviewer A	Tier 2 review day wording	Aligned with [guideline] update
v1.2	Superseded	02 Oct 2026	Dr. Maya	Added diet node (N17)	Diet guidance was missing
v1.1	Superseded	20 Sep 2026	Reviewer A	Split extent and severity tests	Needed separate branches
v1.0	Superseded	08 Sep 2026	Clinical editor	Initial IBD pack	Seeded from [sources]

18 - Benchmark results

Decision support, not diagnosis. 20 synthetic cases. A recommendation counts as grounded only if it links to a graph node and its source.

Grounding benchmark

Graph-grounded agent vs. the same model with no graph · run [run ID] · [date]

SAMPLE DATA — replace with the real run

Cases	Grounded · with graph	Grounded · plain model	Target ≥ 95% grounded
20	**98.7%**	**39.3%**	**✓ PASS**
16 IBD · 4 urology stub	236 of 239 recommendations	94 of 239 recommendations	3 ungrounded lines vs 145 for plain model

Overall grounded share



Dashed line = 95% target

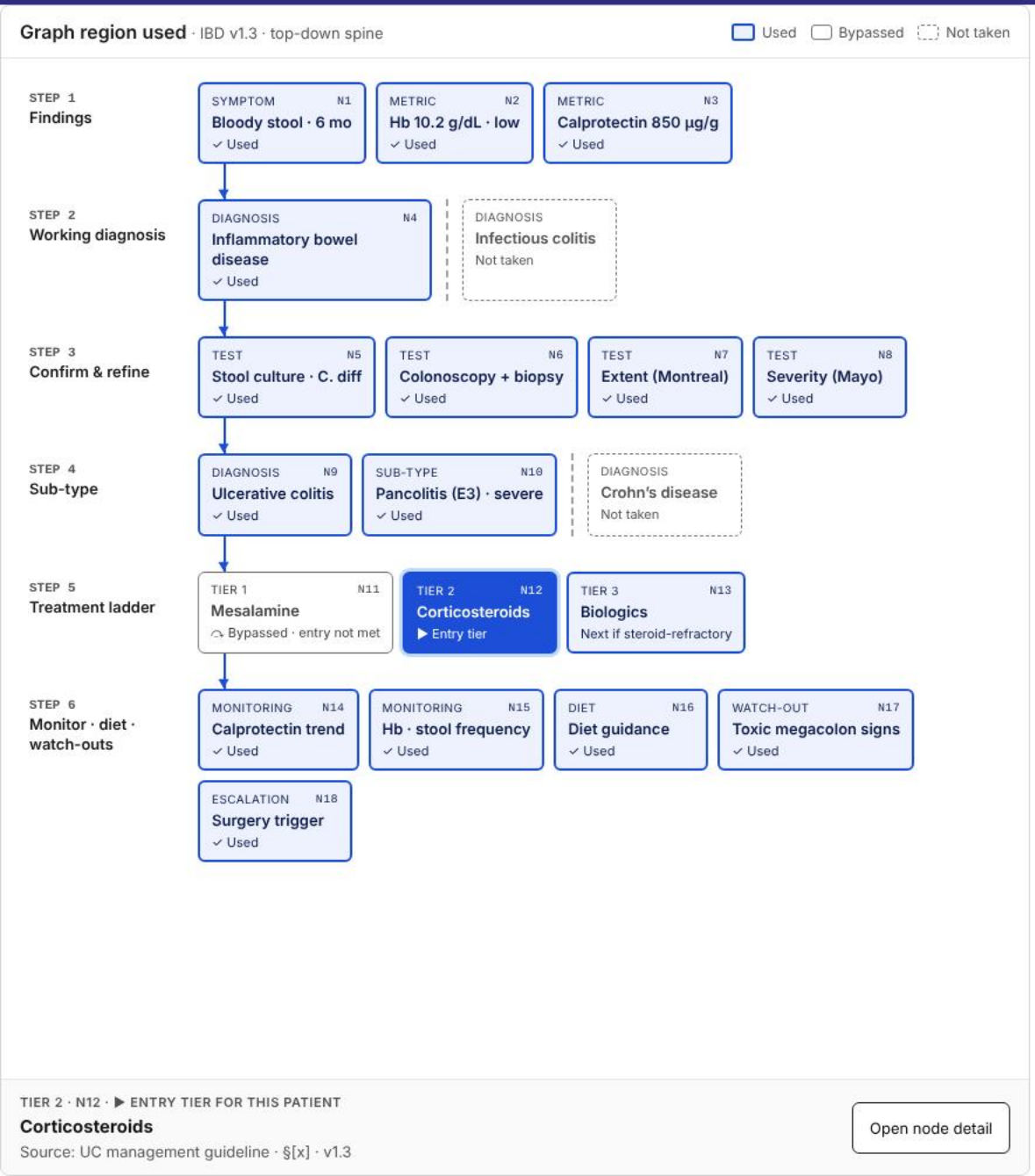
Per case

All 20 | IBD | Urology stub

Case	Specialty	Recs	Grounded: graph (dark) vs plain (light)	Graph	Plain	Result
B-01	IBD	12		100%	41.7%	✓ Pass
B-02	IBD	14		100%	42.9%	✓ Pass
B-03	IBD	10		100%	30%	✓ Pass
B-04	IBD	16		100%	43.8%	✓ Pass
B-05	IBD	12		91.7%	33.3%	✕ Below 95%
B-06	IBD	18		100%	44.4%	✓ Pass
B-07	IBD	11		100%	45.5%	✓ Pass
B-08	IBD	13		100%	30.8%	✓ Pass
B-09	IBD	15		100%	40%	✓ Pass
B-10	IBD	12		100%	41.7%	✓ Pass
B-11	IBD	14		100%	50%	✓ Pass
B-12	IBD	10		100%	30%	✓ Pass
B-13	IBD	17		94.1%	35.3%	✕ Below 95%
B-14	IBD	12		100%	41.7%	✓ Pass
B-15	IBD	13		100%	30.8%	✓ Pass
B-16	IBD	16		100%	43.8%	✓ Pass
U-01	Urology stub	6		100%	33.3%	✓ Pass
U-02	Urology stub	7		100%	42.9%	✓ Pass
U-03	Urology stub	5		80%	40%	✕ Below 95%
U-04	Urology stub	6		100%	33.3%	✓ Pass

Click a case ID to replay it in the workspace (not wired in wireframe) · Method: [how grounding is scored]

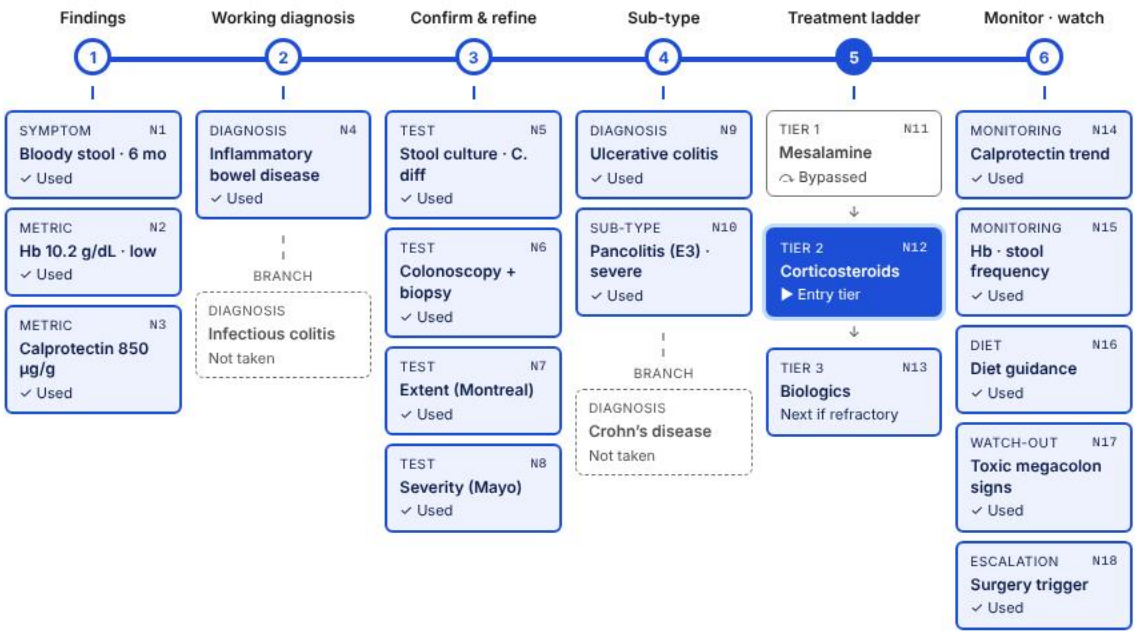
Graph option A - Spine



Graph option B - Metro

Graph region used · IBD v1.3 · metro line

Used · Bypassed · Branch not taken



TIER 2 · N12 · ▶ ENTRY TIER FOR THIS PATIENT

Corticosteroids

Source: UC management guideline · §[x] · v1.3

Open node detail

Graph option C - Focus

Graph region used · IBD v1.3 · focus + neighbours

Click any neighbour to move the focus

N3 Calprotectin 850 µg/g › N4 Inflammatory bowel disease › N6 Colonoscopy + biopsy › N9 Ulcerative colitis ›
N10 Pancolitis (E3) · severe › N12 Corticosteroids › N13 Biologics › N18 Surgery trigger

LED HERE FROM

SUB-TYPE · N10
Pancolitis (E3) · severe
✓ Used

→

TIER 2 N12

Corticosteroids

▶ **Entry tier for this patient**

Induction for moderate to severe disease; IV if severe. Severe disease enters the ladder here. Review response at day [n].

Source	UC management guideline · §[x]
Version	v1.3

Comments (2) Suggest a change

→

LEADS TO

TIER 3 · N13
Biologics
Next if steroid-refractory

MONITORING · N14
Calprotectin trend
✓ Used

MONITORING · N15
Hb · stool frequency
✓ Used

WATCH-OUT · N17
Toxic megacolon signs
✓ Used

ESCALATION · N18
Surgery trigger
✓ Used

OTHER OPTIONS AT THIS STEP

TIER 1 · N11
Mesalamine
Bypassed

Graph option D - Outline

Graph region used · IBD v1.3 · outline

18 used · 1 bypassed · 2 not taken | Hide not-taken | Collapse all

Step 1 Findings — 3 used

- ✓ Bloody stool · 6 mo — Symptom · used — N1
- ✓ Hb 10.2 g/dL · low — Metric · used — N2
- ✓ Calprotectin 850 µg/g — Metric · used — N3

Step 2 Working diagnosis — 1 used · 1 not taken

- ✓ Inflammatory bowel disease — Diagnosis · used — N4
- ○ Infectious colitis — Diagnosis · considered, not taken — −

Step 3 Confirm & refine — 4 used

- ✓ Stool culture · C. diff — Test · used — N5
- ✓ Colonoscopy + biopsy — Test · used — N6
- ✓ Extent (Montreal) — Test · used — N7
- ✓ Severity (Mayo) — Test · used — N8

Step 4 Sub-type — 2 used · 1 not taken

- ✓ Ulcerative colitis — Diagnosis · used — N9
- ○ Crohn's disease — Diagnosis · considered, not taken — −
- ✓ Pancolitis (E3) · severe — Sub-type · used — N10

Step 5 Treatment ladder — 3 used

- ↷ Mesalamine — Tier 1 · bypassed, entry not met — N11
- ▶ **Corticosteroids** — Tier 2 · entry tier for this patient — N12
- ✓ Biologics — Tier 3 · Next if steroid-refractory — N13

Step 6 Monitor · diet · watch-outs — 5 used

- ✓ Calprotectin trend — Monitoring · used — N14
- ✓ Hb · stool frequency — Monitoring · used — N15
- ✓ Diet guidance — Diet · used — N16
- ✓ Toxic megacolon signs — Watch-out · used — N17
- ✓ Surgery trigger — Escalation · used — N18

TIER 2 · N12 · ▶ ENTRY TIER FOR THIS PATIENT

Corticosteroids

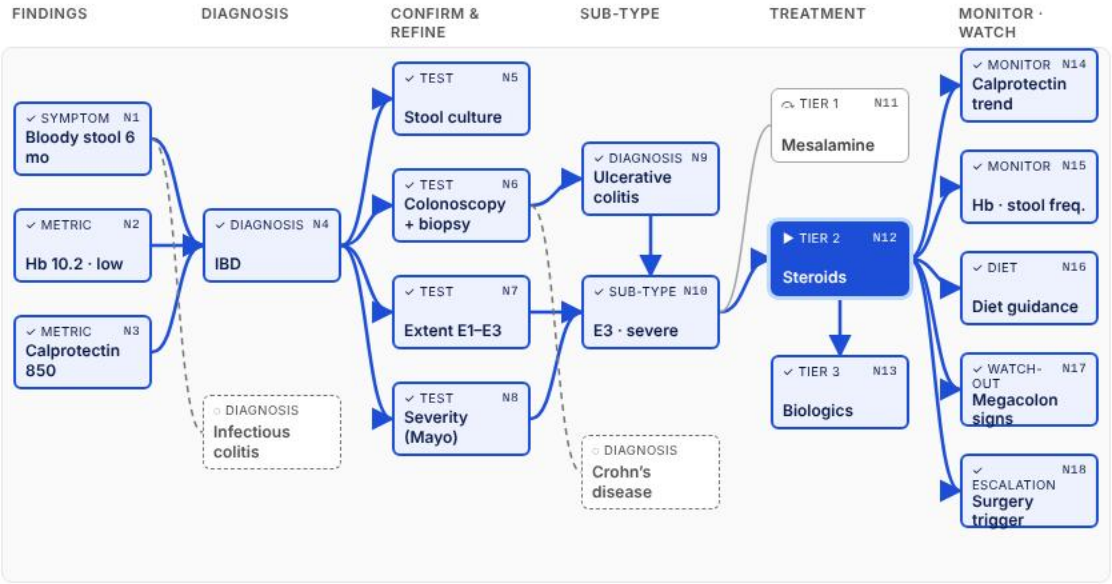
Source: UC management guideline · §[x] · v1.3

Open node detail

Graph option E - Network

Graph region used · IBD v1.3 · network map

Path walked — Bypassed ----- Not taken



Fixed layered layout: columns are walk stages, so the map never reshuffles between sessions. Zoom and pan for larger packs.

TIER 2 · N12 · ▶ ENTRY TIER FOR THIS PATIENT

Tier 2 · Corticosteroids

Source: UC management guideline · §[x] · v1.3

Open node detail